MANAGED CARE · UNITED STATES · NYSE: CNC

Centene

Cheap for the wrong reason: the screen read stale earnings, and the recovery came from elsewhere

NEUTRAL | Case study: value trap | 12 months

THE CALL INTRADAY, 29 OCT 2026	
Rating	NEUTRAL
Price	$67.60
Base-case value P/E on 2027 adjusted EPS	$65.00
Upside to base case	−4%
Market cap	$33.5bn
52-week range	$31.63 – $72.50

01 Overview

Centene is in this pitch set as a case study. It shows the main weakness of a value screen that uses lagged accounts: when earnings break fast, trailing multiples fall faster than the price only because the earnings are stale. Centene ranked first of 108 health-care stocks in December 2025 and was held at all ten rebalances. This note explains how a managed-care insurer makes money, what went wrong in 2025 and what the episode teaches.

> In December 2025 Centene was the cheapest health-care stock in our universe, on FY2024 earnings that no longer existed. 2025 adjusted EPS collapsed to $2.08 before a 2026 repricing rebound to a guided floor above $5.20. The stock has recovered; at $67.60 it is fairly priced, and the lesson is about process.

MARKET CAP	2026 ADJ. EPS GUIDE	P/E ON THE GUIDE	MEMBERSHIP, Y/Y
$33.5bn	> $5.20	13.0x	−8%

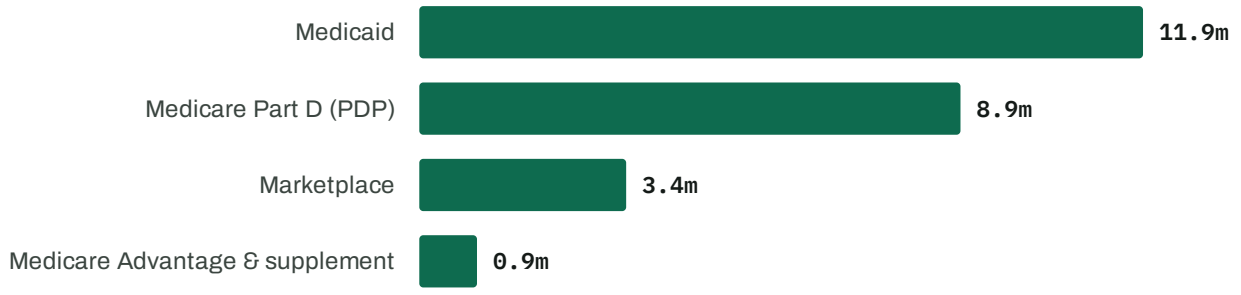
02 The business

Centene is a health insurer paid by governments and individuals to cover medical costs. It receives a premium per member and pays the doctors, hospitals and pharmacies. The key number is the health benefits ratio (HBR): medical costs divided by premiums. At 90% the insurer keeps 10 cents per dollar to cover admin costs and profit; at 93% there is almost nothing left.

Medicaid (health cover for low-income Americans, run by states) is the core: 11.9m members. The Marketplace (Ambetter) sells subsidised individual plans under the Affordable Care Act. Medicare covers people over 65, mostly through stand-alone drug plans (Part D) at Centene.

Two technical mechanisms drive surprises. Risk adjustment moves money between insurers depending on how sick their members are, and the data arrives months after the year. State Medicaid rates are reset with a lag, so when costs jump the insurer absorbs them for a while.

Members by product, Sep 2026



Medicaid

Largest US Medicaid managed-care organisation: 11.9m members at September 2026, down from 12.7m.

Marketplace (ACA)

Ambetter plans: 3.4m members versus 5.8m a year earlier after enhanced premium tax credits expired.

Medicare

Wellcare: 0.95m Medicare Advantage and supplement members plus 8.9m stand-alone Part D members.

Other

Remaining Magellan Health businesses held for sale ($513m impairment, December 2025).

03 Industry & macro

US health policy moved against Centene in 2025-2026. The enhanced ACA premium tax credits expired at the end of 2025, and Marketplace membership fell from 5.5m to 3.4m. Healthier members tend to leave first, which worsens the risk pool.

The One Big Beautiful Bill Act (July 2025) brings Medicaid work requirements and more frequent eligibility checks from end-2026. Fewer members means less revenue, and the members who remain are usually sicker.

Medical cost trends rose across the industry: behavioural health, home health and expensive drugs. On the positive side, the final 2027 Medicare Advantage rate notice (+2.48%) was much better than the January proposal.

ACA SUBSIDIES

−42%

Marketplace membership y/y after enhanced premium tax credits expired on 31 Dec 2025.

MEDICAID REFORM

2027

OBBBA work requirements and more frequent eligibility checks phase in from end-2026.

MEDICARE RATES

+2.48%

CMS final 2027 Medicare Advantage rate notice (April 2026), versus +0.09% proposed.

MEDICAL COST TREND

93.6%

Medicaid benefits ratio in Q3 2026: behavioural health, home health and high-cost drugs.

04 What happened

The events that shaped the stock, in order.

1 Jul 2025
2025 guidance withdrawn: Marketplace risk adjustment ~$1.8bn worse; Medicaid trend up

Q3 2025
$6.7bn goodwill impairment

31 Dec 2025
Enhanced ACA premium tax credits expire

6 Feb 2026
FY2025 adjusted EPS $2.08; 2026 floor above $3.00

28 Apr 2026
Floor raised to $3.40

28 Jul 2026
Floor raised to $4.80 (incl. ~$0.50 non-recurring)

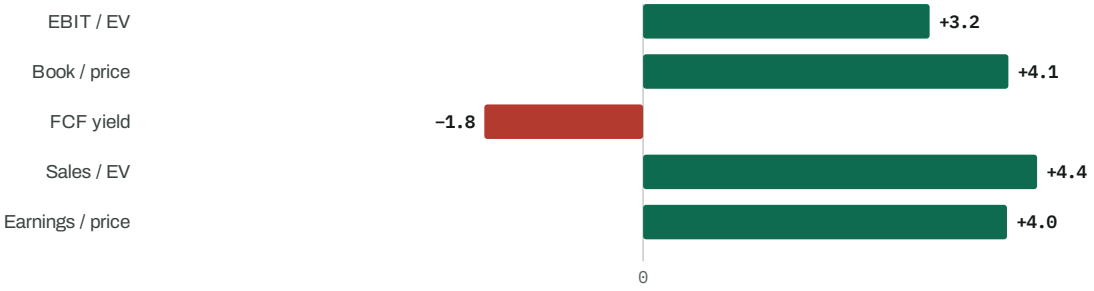
27 Oct 2026
Q3 beat ($0.58 vs ~$0.10 consensus); floor raised to $5.20

05 Why the screen picked it

On the December 2025 score Centene had z-scores of +4.1 on book-to-price, +4.0 on earnings-to-price, +4.4 on sales-to-EV and +3.2 on EBIT-to-EV, but −1.8 on FCF yield. The lag rule meant FY2024 accounts were used, before the 2025 collapse. EV/Sales is structurally low for any insurer (about $195bn of revenue on a $33bn market cap) and says nothing about margins. A revision or quality filter (estimate momentum, HBR trend, goodwill-to-equity) would have flagged the break.

VALUE SCORE, DEC 2025	ACCOUNTS USED	WEIGHT, DEC 2025	H1 2026 RETURN
+2.47 rank 1 of 108 in Health Care	**FY to 2024-12** year-end + 6-month lag	**2.0% / 1.7% / 0.5%** equal · min-var · mean-var	**+62.0%** benchmark +10.8%

Z-scores within sector · Dec 2025 rebalance · above 0 = cheaper than the sector average



MULTIPLE, DEC 2025	CENTENE	PEER MEDIAN	SECTOR MEDIAN
EV/EBIT	6.81x	9.85x	22.71x
P/B	0.68x	1.66x	4.23x
P/FCF	—	13.31x	20.95x
EV/Sales	0.13x	0.39x	4.54x
P/E	5.64x	19.73x	28.10x

Multiples on the accounts the model could use at the time. Peer set: same GICS industry group in the developed universe. Held at: 2021-06, 2021-12, 2022-06, 2022-12, 2023-06, 2023-12, 2024-06, 2024-12, 2025-06, 2025-12.

06 Investment thesis

01 Why the model bought it

With a six-month accounting lag, the December 2025 score used FY2024 accounts: pre-collapse EBIT and book equity before a $6.7bn goodwill impairment. EV/EBIT, P/B and E/P all screened at the top of the sector.

02 What actually happened

In July 2025 Centene withdrew guidance on Marketplace risk adjustment and Medicaid cost trends. 2025 adjusted EPS came in at $2.08. Repricing then worked: the 2026 floor was raised four times, from $3.00 to $5.20.

03 Where it stands now

At $67.60 the stock trades at 13x a 2026 figure that includes about $0.50 of non-recurring items. Membership is shrinking (−8% y/y) and Medicaid still runs at a 93.6% benefits ratio. Upside and downside look balanced.

07 Variant perception

Where our reading differs from the obvious one.

WHAT THE MARKET SAYS	WHAT THE EVIDENCE SAYS
“The stock has doubled, so the model was right.”	The 2026 rebound came from repricing and policy, not from the cheapness the model measured. The signal had no defence against a fast fundamental break.
“Managed care is a stable compounder.”	Margins are thin: each 10 bp of benefits ratio on ~$175bn of premiums is about $175m of pre-tax profit.
“Guidance raises mean upside.”	The floor format ('greater than $5.20') and ~$0.50 of non-recurring items flatter 2026; FY2027 guidance in February is the real test.

08 Financials

LINE	FY2025	9M 2026	Q3 2026	2026 GUIDE
Total revenues	$194.8bn	$154.2bn	$50.6bn	$196.5-200.5bn
Health benefits ratio	91.9%	89.9%	92.7%	90.4-91.1%
Adjusted diluted EPS	$2.08	$6.45	$0.58	> $5.20
GAAP diluted EPS	−$13.53*	$5.15	−$0.14	> $3.46
Total debt	$17.4bn	—	$15.5bn	—
Debt / capital	46.5%	—	41.1%	—

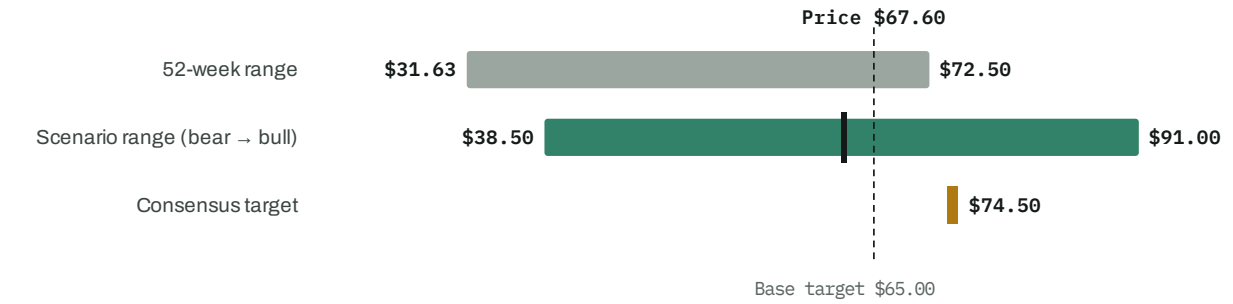
*Includes a $6.7bn non-cash goodwill impairment (Q3 2025). No dividend. Sources: Centene FY2025 and Q3 2026 releases.

09 Valuation

Method: P/E on 2027 adjusted EPS. Managed-care insurers are valued on earnings. We apply peer-range P/E multiples to three 2027 adjusted EPS outcomes: a reset below the 2026 floor ex one-offs (~$4.70), a flat year, and continued margin repair.

SCENARIO	INPUTS	VALUE / SHARE	VS PRICE	WHAT HAS TO HAPPEN
Bear	EPS $3.50 × 11x	$38.50	−43%	Medicaid margins stay under water; risk-adjustment shock
Base	**EPS $5.00 × 13x**	**$65.00**	**−4%**	**2026 earnings power holds ex one-offs**
Bull	EPS $6.50 × 14x	$91.00	+35%	Medicaid rates catch up; buybacks resume

Valuation range · base case marked



Consensus target: $74.50 (+10%). Rating thresholds: Overweight above +15%, Neutral –10% to +15%, Underweight below –10%.

10 Catalysts

Nov 2026-Jan 2027
ACA open enrolment for 2027 (median premiums +15%)

Feb 2027
Q4 2026 results and first FY2027 guidance

From Jan 2027
Medicaid work requirements and eligibility checks

Apr 2027
Final 2028 Medicare Advantage rate notice

Jun-Jul 2027
Risk-adjustment data for benefit year 2026

11 Risks & sell rules

RISK	MITIGANT
Marketplace risk pool worsens as healthier members lapse	Repricing already filed; membership smaller
Medicaid rate adequacy versus acuity	State rate increases in 2026
Low Medicare Star ratings limit bonuses	Small MA book (0.95m members)
Little unregulated cash ($175m)	Debt reduced from $17.4bn to $15.5bn

WE CHANGE THE CALL IF

- FY2027 guidance materially below ~$4.70 of adjusted EPS
- Medicaid benefits ratio failing to improve from ~93.5%
- Another mid-year risk-adjustment shock

The bear case, taken seriously

Even after the recovery, the bear case is credible: membership is shrinking, Medicaid margins are negative on an HBR basis, 2026 includes one-offs, and only $175m of cash sits outside regulated subsidiaries.

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12 Interview prep

Questions a portfolio manager is likely to ask, with short answers.

Q Your model held Centene through a 2025 collapse. What does that tell you?

That lagged trailing multiples cannot see a fast fundamental break. The fix is to add a quality or revision signal, not to drop value.

Q The stock doubled from its low. Wasn't the model right?

The outcome was good, but for reasons the model did not measure: repricing and policy. I judge the process, not the outcome.

Q Why Neutral now?

13x a 2026 floor that includes one-offs, shrinking membership and no 2027 guidance: upside and downside look balanced.

Q What one metric would you watch?

The Medicaid benefits ratio. Below about 92% the earnings recovery is real; stuck near 93.5% it is not.

13 Glossary

Health benefits ratio (HBR)	Medical costs divided by premium revenue; the core margin measure for an insurer.
Risk adjustment	Transfers between insurers based on members' health status, settled after the plan year.
Medicaid / Medicare	US public health programmes for low-income people (states) and for people over 65 (federal).
Guidance floor	A 'greater than' figure: the company commits to a minimum, not a midpoint.

14 Sources

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Company figures from filings and releases listed above. Screen data (multiples, z-scores, weights) from the Bloomberg extract of 26-29 October 2026, aggregated. Peers: Elevance Health, Humana, Cigna, UnitedHealth, CVS Health (pipeline peer set).

Student research for educational and interview purposes. Not investment advice. Prices as of intraday, 29 Oct 2026.